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OSUN STATE UNIVERSITY, OSOGBO

NSG 509 — Past Questions & Materials

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Course: NSG 509

Institution: Osun State University, Osogbo

Papers in this pack: 1

Levels: 500

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Paper 1 of 1 — INTRODUCTION TO HEALTH ECONOMICS COURSE

CODE: NSG 509

Level 500 • First Semester • Handout

INTRODUCTION TO HEALTH ECONOMICS

COURSE CODE: NSG 509

2022/2023 SESSION

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COURSE DESCRIPTION

• The course is designed to expose learners to economic and health principles as they inform health care availability, accessibility and provision as well as methods of covering health risks, use of insurance in health care, management and regulation of hospital costs, issues in managed and long term care.

COURSE OBJECTIVES

- To define and understand the basic concepts in economics
- To elaborate the concept of health economics
- To describe the basic concepts in health economics
- To describe the relationship between health and economy
- To describe the elements of health care cost
- To describe the reasons for the increasing cost of health care
- To explain the concept of budgeting in healthcare
- To explain the concept of cost containment in healthcare
- To describe various options of healthcare financing
- To describe the concept of health

insurance

- To describe the operation of National health insurance scheme

COURSE SCHEDULE

• INTRODUCTION TO ECONOMICS (week 1)

- Definition of economics
- Basic concepts in economics
- Demand and supply
- Economic Systems

• INTRODUCTION TO HEALTH ECONOMICS

(week 2 – 3)

- Definition of Health Economics
- Why health economics
- Economy and Health

• COST OF HEALTH CARE (week 4 – 5)

- Types of Health Care Cost
- Elements of Cost
- Benefits from Health Care
- Economic efficiency in health care
- Reasons for the Present Trend in Cost of Health Care

• HEALTHCARE BUDGETING (week 6 – 7)

- Definition of Budget
- Basic Concepts related To Budget
- Types of Budget
- Advantages of budgeting
- Disadvantages of budgeting

• COST CONTAINMENT IN HEALTH CARE

(week 8)

- Elements of cost containment
- Ways to Contain Cost of Health Care
- HEALTH CARE FINANCING (week 9 – 10)
- Reasons for increasing cost of health care
- Options in Health Care Financing
 - Direct Government Financing
 - User Charges (Out-Of-Pocket Expenses)
 - Community Financing
 - Health Insurance
 - Foreign Aid
 - Voluntary contributions

- HEALTH INSURANCE (week 11)
- De nition
- Types of Health Insurance
- Reimbursement
- Problems Of Health Insurance
- NATIONAL HEALTH INSURANCE SCHEME IN NIGERIA (week 12)
- Objectives of NHIS scheme
- Health Care Bene ts of NHIS
- How the Scheme Works
- Intended bene ciaries
- REVISION (Week 13)

INTRODUCTION TO ECONOMICS

(Week 1)

De nition of Economics

- ere are several de nitions of health economics.
- Study of how we use scarce resources to produce goods and se ices to satisfy our wants
- Social science concerned with the e cient use of limited or scarce resources to achieve maximum satisfaction of human material wants
- e study of how best to allocate scarce resources among competing ones

Introduction cont;

- De nition of Health Economics
- Application of the theories, concepts, and techniques of economics to the health sector
- Concerned with issues like;
 - Allocation of resources within the various health care strategies
 - Quantity and quality of resources used in health care delive
 - Funding of health care se ices
 - E cient use of resources allocated for health care and the e cts of preventive, curative, and rehabilitative health se ices on individuals and the society

Introduction cont;

Health is a state of complete physical and mental well-being and not merely the absence of disease or infirmity (WHO, 1948) From this definition some people will still add and good spiritual wellbeing

- In all, the aim is to have:
- A life free of disease
- Free of physical disability
- Free of mental impairment
- Free of social shaming
- Free of psychological impairment.
- Health economics is defined as a branch of economics that concerns with the study of management of health care resources in providing balanced health care.
- e resources include men, materials, machines, finance and information.
- Health economics is the study of how (scarce) resources are allocated to and within the health economy.

Why health economics:

- Increasing people's healthcare needs and demand
- Renewed cost-consciousness within the health system
- e shift from exclusively humanistic approach to increasing use of managerial techniques and quantitative research methods
- Increasing costs of medical care (due to heavy disease burden, technological changes and cost of required health care inputs)
- e problem of scarcity

Why health economics cont;

- e size of the health economy is large and growing.
- Role of government in the health care markets
- Health care market is different from other markets

Basic Concepts in Economics

- Good: these are the outputs (such as

health care) of a production process that involves the combining of different resources such as labour and equipment.

- Goods (including services) are valuable in the sense that they provide some utility (see below) to individual consumers. They are termed 'goods' as they are desirable, as distinct from 'bads' which you will read about later!

Basic Concepts in Economics

cont;

- Service: an intangible action that is capable of satisfying human want – such as health care
- Goods are used to provide services
- Scarcity: the central concept in economics which describes a condition in which it is impossible to satisfy all human wants for goods and services. Hence need for making choice.

Basic Concepts in Economics

cont;

- Opportunity cost (economic cost). As resources are scarce, an individual, in choosing to consume a good, in principle, chooses the good which gives him or her the greatest benefit, and thus forgoes the consumption of a range of alternative goods of lesser value.
- The opportunity cost is the value of the benefit of the next best alternative

Basic Concepts in Economics

cont;

- CHOICE: There are two ways in which society can make choices about the allocation of resources to production in the health sector, and the distribution of the services that are produced among those that want them.

- A society can leave these decisions to the market – letting demand, supply and prices determine resource allocation, or it can plan, usually by giving its government.

Basic Concepts in Economics

cont;

- Utility: the happiness or satisfaction an individual gains from consuming a good. The more utility an individual derives from the consumption of a good, all else being equal, the more they would be willing to spend their income on it.
- Marginal utility: the additional utility gained by consuming one more unit
- Economic resources: all natural, human, and manufactured resources which go into the production of goods and services
- It is broadly divided into two:
 - Property resources include land (natural resources) or raw materials and capital
 - Human resources include labour and entrepreneurial ability

Basic Concepts in Economics

cont;

- Demand and Supply
- Demand: the quantity of a product that consumers will purchase at each possible price. Demand is related to price because it is the price that determines the quantity
- Demand means ability to pay at a given time.
- It is referred to as effective demand.

Generally the relationship between price and quantity is negative.

- It means that the higher the price level the lower will be the quantity demanded and, the lower the price the higher will be the quantity demanded.

LAW OF DEMAND

As the price rises,
the quantity demanded falls.

Demand

Price Quantity Demand for ____

6

N5 10 5

N4 20 4

ecirP

3

N3 30 2

N2 40 1

0

10 20 30 40 50

N1 50 Quantity

Basic Concepts in Economics

cont;

- Determinants of demand
- Income
- Price to relative goods: compliments and substitutes
- Fear of rise in price
- Ostentation goods e.g gold, satellite disk which their demand will not reduce even when their price go up.
- Tastes and preferences (Personal feelings toward the value or desirability)
- Disposable income (amount of income that people have left after they pay their taxes)

Basic Concepts in Economics

cont;

- Changes in fashion
- Increase income
- Taxation
- Increased population.
- Price of related goods
- Number of consumer

Cont;

- Supply: the quantity of a good or service that firms will offer for sale at each possible price
- Price affect supply.
- An increase in price will result in producers wanting to increase the quantity of a

product they will sell on the market (chasing profits). Therefore the relationship between the price and supply is positive – the higher the price the higher the quantity supplied

LAW OF SUPPLY

As the price rises,
the quantity supplied rises.

SUPPLY

• Price Supply of ____

Price Quantity

• Quantity

6

N5 50 5

4

N4 40

ecirP

3

N3 30 2

1

N2 20

0

10 20 30 40 50

N1 10 Quantity

Basic determinants of supply:

- Resource prices/income
- Technique of production
- Taxes and subsidies
- Prices of other goods
- Price expectations
- Number of sellers in the market
- Development of new method
- Weather condition

• TYPES OF SUPPLY

- Competitive
- Joint or complimentary supply
- composite supply
- READ ABOUT TYPES OF DEMAND AND SUPPLY

Supply Cont;

- Reasons Why Health Care need

exceeds effective demand:

- Affordability
- Geographical accessibility
- Availability
- Acceptability
- Cost of time off from work and waiting

Supply constraints;

- Economic Systems
- Set of rules or understandings that govern how scarce resources are used to produce goods and services that satisfy human wants
- Not isolated from the political and social organization or system

• All nations have economic systems

• All nations are faced with the problem of scarcity

because of limited resources

• Scarcity forces every economic system to address

three central economic questions which are:

- What goods and services will be produced from our scarce resources?
- How will goods and services be produced?
- For whom will goods and services be produced?

• Capitalism:

• An economic system in which the factors of production are owned and controlled by the people

• It is profit-oriented

• Role of government in market economy is limited

• Breeds competition

• Private clinics exist to make profits and are privately owned by individuals or groups

• Socialism:

• An economic system in which the government owns and controls the factors of production

• Workers are ensured employment

• Goods and services are shared regardless of the value of their contribution to production

- Communism:
- An economic and political system that combines government ownership and control of the factors of production with a totalitarian form of government

Mixed economy:

- Capitalism and socialism as economic systems
- Commonly the case in many countries

PRACTICE QUESTIONS

- Define demand and supply
- Briefly discuss the variables that determine demand for healthcare services in Nigeria
- List and briefly explain various economic systems available worldwide

THE PRODUCTION OF HEALTH

Health as a Durable Good

- Consumer demand is based on goods and services that create utility.
- In this case, health creates happiness, rather than the actual services that add to health.
- The demand for health care is derived from the underlying demand for health itself.
- Health can be considered to be a durable good.
- Each person has an inherent “stock” of health.
- Utility = $U(X, H)$, where X is a bundle of other goods, and H is the (unobservable) stock of health.
- Assume that more health is better.

Cont:

- For every possible medical intervention, there is some point where the marginal productivity of medical care will become very low or negative.
- Important to distinguish between average and marginal effects
- It is overly simplistic to think of a single medical care production function.
- Some medical interventions do not change the eventual level of health of the person, but can speed up the process of a “cure.”
- A cut will heal on its own, but will heal faster with bandages and

antibiotic ointment.

- Lifestyle and other factors also have effects on the productivity of medical intervention.
- Our stock of health wears out over time (depreciation).
- A typical person's health stock might look like this Figure, increasing during childhood, gradually declining with age,
- and punctuated by random events.
- Path also depends on medical technology, as events that might have caused death in earlier periods.
- At HMIN, stock of health care is so low that the person dies.
- Economy and Health:
- Health plays a major role in the socio-economic development
- Economic development requires a healthy workforce
- Healthy people are more productive
- Economic development usually impact on the health of the population (positive and negative)

PRACTICE QUESTION

- Briefly describe the relationship between health and economy

COST OF HEALTH CARE (Weeks 4 – 5)

- Involves 'prices' and 'quantities' spent on healthcare
- Types of Health Care Cost:
- Direct medical cost: Medical cost incurred for medical products and services
- Used to prevent, detect, and or treat a disease
- Covers costs for drugs, laboratory tests and supplies
- Usually have monetary value

Cost of health care Cont;

- Direct non – medical cost: Type of cost cover non-medical services that results from illness but do not involve purchasing medical services
- Example include cost of transportation, food, family care
- Indirect non-medical cost: Cost result from reduced productivity because of ill-health
- Refers to patient's inability to do his usual job (loss of productivity and income)
- Intangible costs: Non – financial outcomes of disease and medical care
- Usually cannot be expressed or estimated in monetary value
- Can be in form of suffering, pain and grief

cont;

- Elements of Cost
- Cost comprises three (3) elements;
- Loss of production
- Expenditures for medical care
- Pain, discomfort and suffering that accompany the disease

Cost;

- Benefits from Health Care
- It could be psychic or monetary
- They include:
- Relief from pain, suffering, anxiety etc.
- In the form of capital good being monetary "pay off", measured by increased production

Cost;

- Economic efficiency in health care
- Provision of necessary care of good quality at minimum cost
- Aim;
- A better economic balance of services by eliminating ineffective, excessive and unnecessary medical procedures
- Reasons for the Present Trend in Cost of Health Care
- Demographic reason
- Population growth is directly related to health

care cost

- Labour intensive nature of health services
 - Skilled Health care labour is directly related to health care cost
- Quality of health services
 - Advances in technology, improved quality of diagnosis and therapy increases effectiveness and healthcare cost
- Public expectation
 - People desire increasing standard of healthcare facilities and high demand for curative health care has increased healthcare cost
- Changing epidemiological picture during socio-economic development
 - chronic and degenerative diseases increases cost of care or cure
- Organization and structure of health system
 - Uncoordinated activities and financing of healthcare agencies and services (especially in preventive health care services)
- Extension of health services coverage
 - Attempts to extend the range, coverage or impact of services to a larger population increase costs

PRACTICE QUESTIONS

- Describe the elements of health care cost
- Describe the reasons for the increasing cost of health care

HEALTHCARE BUDGETING

(Weeks 6 – 7)

- Definition of Budget
- A quantitative statement, usually in monetary terms, of the expectations of a defined area of the organization over a period of time in order to manage financial performance
- A plan for the allocation of resources
- A control for ensuring that results (expressed in quantitative terms) comply with the plans
- Often associated with financial statements (such as revenues and expenses)
- May also be in form of non-financial statements

covering output, materials and equipment

Budgeting cont;

- Basic Concepts related To Budget
- Budgeting:
 - e process of planning and controlling future operations by comparing actual results with planned expectations
- Controlling:
 - e process of comparing actual results with the results projected in the budget
- Incremental (line-by-line) budget:
 - is is a budget worksheet listing expense items on separate lines
 - is is usually divided into sala and non-sala expenses
 - Its simple but discourages cost-e ciency

Budgeting cont;

- Zero-based budget:
 - A budgeta approach that assumes the base for projecting next year's budget is zero
 - Requires managers to justify all activities and proposed expenditure in line with organization's objectives and current environment, regardless of the level of expenditure in previous years
- Fixed budget:
 - A budget in which budgeted amounts are set regardless of changes that occur during the year such as volume of patient, unanticipated in ation
- Variable budget:
 - A budget developed with room for adjustments based on variables such as changes in revenues, patient volume, utilization of supplies and other
- expenses
- Fiscal budget:
 - A speci ed 12-month period during which operational and nancial pe ormance is measured
- Types of Budget
- Operating Budget (Revenue – and – expense budget or annual budget)
 - Organization's statements of expected revenues and expenses for the coming scal year of the organization

- Reveals an input – output analysis of expected and revenues and expenses
- Revenue budget; may represent the patient care income expected for the budget period

Budgeting cont;

- Expense budget; consists of sala and non-sala items
- should be comprehensive, thorough and consider both controllable and non-controllable expenses
- Non-controllable expenses include indirect expenses like lighting, equipment depreciation, probability of rises in materials and labour costs

Budgeting cont;

- Personnel Budget
- Estimate the cost of direct labour necessary to meet the agency's objectives
- Guides recruitment and discharge personnel
- Directly reflects the manager's ability to supervise and lead the staff
- Information helpful in budgeting include;
 - Current staffing pattern
 - Number of vacant positions
 - Previous years reports and performances
 - Variety of patient cases
 - Seasonal variation in patient load and disease burden

Budgeting cont;

- Capital Expenditure Budget
- Important plan to meet the organization's long term goals
- Capital expenditures include physical changes such as replacement or expansion of the plant, major equipment and inventories
- Usually administrators establishes ceiling for capital budget and the nurse manager will need to prioritize requests if they exceed the available fund

Budgeting cont;

- Cash Budgets
 - Planned to make adequate funds available as needed and to use any extra funds profitably i.e. to purchase needed resources
 - If well planned, it will provide cash as needed and produce interest on excess fund
- Flexible Budgets
 - Plan to accommodate unpredictable expenses that occur suddenly
 - e changes can be compensated for by having periodic budget reviews
 - Also, variation in cost can be predicted through historical analysis of costs in previous budgets
- Advantages of budgeting
- Plans for detailed programme activities
- Fix accountability by assignment of responsibility and authority
- State goals for all units, offer a standard of performance and stress the nature of the planning and control process
- Encourage careful analysis of operations and evidence – based decisions
- Minimize hasty judgments in decision making
- Can expose organizational weaknesses and allow corrective measures
- Resources can be projected and waste minimized
- Financial matters can be handled in orderly fashion and activities of organizations can be coordinated and balanced

Budgeting cont;

- Disadvantages of budgeting
- Only focuses on aspects of organization activities that are easily measured
- Convey all aspects of organization performance into monetary values
- May become an end in itself instead of a means to an end
- Budgetary goals may sometime supersede the organization's goals and gain autocratic control of the organization
- Danger of over-budgeting making the budget cumbersome and expensive
- Time consuming

- Requires skill and experience for successful control
- Requires forecasting which can be uncertain because budgetary control is subject to human judgment, interpretation and evaluation

PRACTICE QUESTIONS

- Define the term budget
- Write short notes on the types of budget
- List the advantages and disadvantages of budget

COST CONTAINMENT IN HEALTH CARE

(Week 8)

- The goal is to keep costs within acceptable limits for volume, intensity and other parameters
- It involves;
 - Costs awareness,
 - Costs monitoring,
 - Costs management
 - Incentives to prevent, reduce and control costs.

Elements of cost containment

- Cost Awareness
 - Focuses the health staff's attention on costs of service delivery and the steps available for containing them
- Cost Monitoring
 - Answers questions like how, when, where and why "is to be spent"
 - Ultimate aim at checking where wastes can be reduced
 - Requires ingenuity of staff's ideas and demonstration of money-saving measures in their unit

Elements of cost containment

- Cost Avoidance
 - Identifying unnecessary procedures to minimize expenditures on the part of the consumer and the organization.
- Cost Reduction

- Includes health care delivery desirable to contain health care cost
- Preventive measures like childhood immunization can save a lot of cost in health care

Elements of cost containment
cont;

- Cost Control
 - Effective use of available resources through careful forecasting, planning, budgeting, reporting and monitoring
- Cost-effectiveness
 - Entails comparing costs and identifying the most beneficial outcomes
 - Can be done by analyzing the alternative methods in achieving the same objective and then determine the cost implication of all inputs for each method

Elements of cost containment
cont;

- Ways to Contain Cost of Health Care
- Rational use of health care service
- Effective referral system
- Financial disincentives to discourage unnecessary use of secondary and tertiary health care
- Appropriate degree of technical complexity in the service delivery
- Appropriate utilization of skilled and unskilled healthcare staff
- Positive health behaviour to maintain better health
- Standardization of construction technology, equipments and drugs to the minimum acceptable standard and relatively inexpensive level eg rational prescription and use of drugs
- Using all resource to full capacity to avoid waste
- Economy in procurement of resources of given characteristics

PRACTICE QUESTIONS

- Write short notes on the following;
 - Cost awareness
 - Cost monitoring
 - Cost avoidance
 - Cost reduction
 - Cost control

HEALTH CARE FINANCING

(Weeks 9 - 10)

- Reasons for increasing cost of health care
- Global economic recession
- Aging population with increased health care needs
- Increased use of technology
- New and expensive treatment modalities
- Increasing administrative costs

HEALTH CARE FINANCING cont;

- Options in Health Care Financing
- Direct Government Financing
- e most widespread approach in the developing world
- WHO recommends at least 15% of their total budgeta expenditure to health care at all levels of government
- rough;
 - Periodic allocations from general revenues or
 - Proceeds of a designated tax

HEALTH CARE FINANCING cont;

- Reasons why Direct Government Financing has failed:
 - Economic condition
 - Structural Adjustment programme (cutbacks in government expenditure on social se ices)
 - Immediate exigency such as insecurity

HEALTH CARE FINANCING cont;

- User Charges (Out-Of-Pocket Expenses)
- Another way of nancing health care is by charging patients
- ese charges take a variety of forms
 - A single encounter may be broken into items like laborato test, drugs,
 - procedures, etc.
 - A uniform priced charged for all the patients with exception for the poor, aged and children

– A sliding scale of rates based on persons means to pay

HEALTH CARE FINANCING cont;

- Advantages :
 - Promotes patients' conscious of the services they demand
 - Strengthens self – caring attitudes
 - Foster some level contribution to the health financing.
 - Keep services running and improves quality of care and confidence in the services
- Disadvantages:
 - Fees collection and management requires management capabilities which may not be available at all levels of health care delivery
 - Revenues collected in some may not be substantial compared to cost of providing services
 - May reduce utilization rates

HEALTH CARE FINANCING cont;

- Community Financing
- Community support on providing resources, either financial or material and human to establish or improve health
- Requires community organization
- Sustaining contributions may be problematic as people may be unwilling to continue to pay for programs especially if they are not directly benefiting from it

HEALTH CARE FINANCING cont;

- Dimensions of community financing:
 - Paying at full or preferential rates for health facilities organized through community efforts
 - Paying for socially organized voluntary community insurance schemes
 - Giving of gifts in cash or kind for which no wholly individual benefit is expected but from which the donor may partake of the collective benefits
 - Paying for the creation and utilization of community capitalization schemes for the promotion of health care such as nutrition and sanitation funds from which grants or loans are

given to members for health related activities

HEALTH CARE FINANCING cont;

- Health Insurance
- A system in which prospective consumers of care pay to a third party in the form of an insurance scheme, which in the event of future illness will pay the provider of care for some or all of the expenses incurred
- Health insurance is a mixed source of finance ie often draws contributions from both employers and employees (sometimes government)
- Contributions to such schemes are often mandatory

HEALTH CARE FINANCING cont;

- Types of Health Insurance:
 - Government or social insurance;
 - maybe compulsory or voluntary
 - Often covers people employed in the formal sector
 - Contributions based on individuals income not on actual risk
 - Private insurance;
 - Coverage through third-party payer institutions
 - Employer based insurance;
 - Employers or parastatal or private bodies serve as the third party payer or collection agent

HEALTH CARE FINANCING cont;

- Compulsory insurance schemes:
 - Generally financed by employers and or employees contributions calculated as a percentage of pay roll
 - May cover the self-employed
- Voluntary insurance schemes:
 - People may be allowed to be voluntary contributors to a social security scheme, run by government or statutory agencies
 - Alternatively they may insure with profit or non-profit agencies or they may join a group scheme

HEALTH CARE FINANCING cont;

- Foreign Aid
- Donors are important financiers of health care in Africa
- Generated mostly through development – oriented institutions such as bilateral agencies, multilateral organizations and banks e.g. UNICEF, WHO, UNDP, World Bank, EEC, and USAID etc.
- Generally channeled through a central authority in the recipient country such as Minister of Finance /National Planning
- In some cases, funds may be routed directly to particular agencies or NGOs
- Negative effects:
 - Emphasis on vertical programmes
 - Sustainability of the problem
 - Priority program often determined by donors and not recipient countries
 - Some donor funding of programs are out of proportion to total health needs
 - Poor coordination of efforts by various external agencies involved in funding of the programmes
- Voluntary contributions
- Usually from individuals or groups within the country
- Philanthropists may make cash and/or donations in kind (buildings, equipments, etc)
- Religious groups also fall into this category
- Some groups run non-profit making health services

PRACTICE QUESTIONS

- Describe the major options of healthcare financing

HEALTH INSURANCE

(Week 11)

- Definition
- A system in which prospective consumers of care make payment to a third party in the form of an insurance

scheme, which in the event of future illness will pay the provider of care for some or all of the expenses incurred

HEALTH CARE FINANCING cont;

- A type of insurance whereby the insurer pays the medical costs of the insured if the insured becomes sick due to covered causes, or due to accidents
- e insurer may be a private organization or a government agency
- Health insurance is an agreement between a person, who is called the policy holder, and an insurance agent
- Insurance agents or carriers are organizations that offer financial protection in case of illness or injury and pays for the policyholder's medical treatment
- Types of Health Insurance
- Private Health Insurance
- A contract between an insurance company and the customer and in the private sector
- Can be for groups like companies, labour unions, professional association or for individuals.
- Public Health Insurance
- Public sector third party may be Parastatals, insurance scheme, government and social security and sometimes the providers
- All receive coverage without regard to health status
- Eliminates the problem of adverse selection (tendency for only those who will benefit from insurance to buy it or participate in it)
- Amplifies the problem of moral hazard (the state of mind and change in behaviour that results from the knowledge the health insurance will take care of medical bills)

HEALTH CARE FINANCING cont;

- Social Insurance
- Insurance program financed by government through tax

revenues

- Guarantee citizens financial benefits for events beyond individual control, such as old age, disability and poor health
- Payment is irrespective of the needs and is usually based on employment and income
- Based on the principle of solidarity
- Contribution based on ability to pay
- Resources are pooled together among a large population
- It enhances security of each individual in the group
- Higher income earners will subsidize those with lower income and those with lower health needs will subsidize those with higher health needs

HEALTH CARE FINANCING cont;

- Community Sponsored Insurance
- A community based program which normally operates in the rural areas and mostly localized
- Direct
- Health Insurance Scheme builds or rents its own health care premises exclusively for the use of the insured persons.
- Indirect
- Scheme makes contracts with selected providers for the provision of defined services at negotiated prices, the authority rather than the insured persons makes the payment

HEALTH CARE FINANCING cont;

- Reimbursement
- e patient buys his own medical care in the private market
- He then sends the receipted bills to the insured
- e insured reimburses the insured person
- Reimbursement is either for part of the full cost or on the basis of standard payment for a particular service (which will normally be well below the prices actually paid)
- Problems Of Health Insurance:
- Increasing cost of health care
- Unequal healthcare cost

- Exclusion of some medical problems
- Health care recipient is not involved in negotiating the cost of care
- Some health care providers have popular and unpopular ways of controlling these costs

PRACTICE QUESTION

- Define health Insurance
- Write briefly on the types of health insurance

NATIONAL HEALTH INSURANCE

SCHEME IN NIGERIA (Week 12)

- Health insurance scheme in Nigeria dates back to over 3 decades
- Was officially launched in 1997
- Implementation is planned in phases commencing with public civil servants
- Private sector involvement is incorporated into the scheme with the use of HMOs
- HMOs collect contributions from participants and also pay providers of services

HEALTH CARE FINANCING cont;

- Objectives of NHIS scheme
- To ensure that every Nigerian has access to good health care services
- Protecting families from the financial hardship of huge medical bills
- To ensure equitable distribution of health care costs among different income groups
- Limiting the rise in the cost of health care services
- To improve and harness private sector participation in the provision of health care services
- To ensure equitable patronage of all levels of health care
- To maintain high standard of health care delivery services within the scheme
- To ensure availability of funds to the health sector for improved services
- To ensure efficiency in health care services
- To ensure adequate distribution of health facilities within the federation

HEALTH CARE FINANCING cont;

- Health Care Bene ts of NHIS
- De ned elements of curative care such as:
 - Out patient attendance
 - Maternity care for up to four bi hs for eve insured person
 - Consultation with de ned range of specialist
 - Hospital care in a public or private hospital in a standard ward during a stated duration of stay, for physical or mental disorders
 - Eye examination and care, excluding tests for and the actual
 - provision of spectacles
 - De ned dental care
- How the Scheme Works
- Contributors will rst register with an NHIS approved Health maintenance Organization (HMO)
 - ereafter register with a prima health care provider of his choice for an approved list of providers supplied HMOs
 - e registered contributor will be issued an Identity card (ID) card with a personal identi cation number
 - In the event of sickness the contributor presents his ID card to his chosen prima health care provider (PCP) for treatment

NHIS CONT;

- A contributor has a right to change his PCP after a minimum period of six months if he is not satis ed with his se ices
- Disputes between actors in the scheme shall be settled by arbitration boards to be set up at state level, whose membership includes representative of NMA; Pharmaceutical Society of Nigeria; e National Association of Nigerian Nurses and Midwives and the public
- e HMO will make payment for se ices rendered to him to the health care provider
- A contributor may be asked to make a small co-payment per prescription at the point of se ice

NHIS CONT;

- A contribution made by the insured person entitles himself or

herself, spouse and four children under the age of 18 years to full health benefits

- However students in school up to the age 25 years qualify as dependants
- Extra contributions will be required for additional dependants
- Contribution to be made by formal sector employees for health benefits under the scheme will be 15% of wages, the payment of which will be by both the employee and the employer
- The employee pays 5%, while the employer makes up the remaining 10%
- The employee's part of the contribution is to be deducted from his pay with the employer adding his own and subsequently forwarding the total payment to the appropriate quarters.

NHIS CON;

- Intended beneficiaries:
- Employers in the formal sector (public and private)
- Self-employed person (market women, traders, artisans, farmers
- and Businessmen etc)
- Rural dwellers
- Vulnerable groups (the unemployed, aged, disabled, street children, retarded and retirees)

PRACTICE QUESTION

- Describe concisely, how the National Health Insurance scheme works

REVISION (Week 13)

FURTHER READING

- Cornacchia, H and Berret S. (1982). Shopping for Health Care. Mosby Press, New York: 4 – 22.
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